PACKAGE INSERT

FENADIUM TABLETS 25MG / FENADIUM F TABLETS 50MG

Each Fenadium Tablets 25mg contains : Diclofenac sodium 25 mg
Each Fenadium F Tablets 50mg contains : Diclofenac sodium 50 mg

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Product Description:
Fenadium Tablets 25mg:
Round, convex, yellow, enteric-film coated tablet without markings.
Fenadium F Tablets 50mg:
Round, convex, beige, enteric-film coated tablets without markings.

Pharmacodynamics:
Diclofenac sodium is a phenylacetic acid derivative which has analgesic, anti-pyretic and anti-inflammatory actions. Its mode of action, like that of other non-steroidal anti-inflammatory agents, is not completely understood, but may be related to prostaglandin synthetase inhibitor. It is usually used in the treatment of rheumatoid arthritis and other rheumatic disorders.

Pharmacokinetics:
Diclofenac is absorbed from the gastro-intestinal tract. Peak plasma concentrations are generally attained one to two hours after administration. At therapeutic concentration, it is more than 99% bound to the plasma proteins. It is metabolised and excreted mainly in the urine. Small amounts are excreted in the bile.

Indication:
It is indicated for inflammatory and degenerative forms of rheumatism; rheumatoid arthritis; ankylosing spondylitis; osteoarthritis and spondyloarthritis; non-articular rheumatism; acute attacks of gout. Symptomatic treatment for primary dysmenorrhoea.

Recommended Dosage:
It is generally given in doses of 25mg to 50mg three times daily with food. As a general recommendation, the dose should be individually adjusted. Adverse effects may be minimized by using the lowest effective dose for shortest duration necessary to control symptoms (see section WARNINGS AND PRECAUTIONS).

Established cardiovascular disease or significant cardiovascular risk factors
Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤100 mg daily if treated for more than 4 weeks (see section WARNINGS AND PRECAUTIONS).

Route of Administration: Oral

Contraindications:
- It should not be used in patients who have previously exhibited hypersensitivity to it or in individuals with the syndrome of nasal polyps, angioedema and bronchospastic reactivity to aspirin or other non-steroidal anti-inflammatory agents.
- Severe cardiac failure (see section WARNINGS AND PRECAUTIONS).

Warnings and Precautions:
<u>Gastrointestinal Effects</u>
NSAIDs, including diclofenac, may be associated with increased risk of gastrointestinal anastomotic leak. Close medical surveillance and caution are recommended when using diclofenac after gastrointestinal surgery.

<u>Hypertension</u>
NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

<u>Cardiovascular Effects</u>
Treatment with NSAIDs including diclofenac, particularly at high dose and in long term, maybe associated with an increased risk of serious cardiovascular thrombotic events (including myocardial infarction and stroke).
Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses ≤100 mg daily when treatment continues for more than 4 weeks.
As the cardiovascular risks of diclofenac may increase with dose and duration of exposure, the lowest effective daily dose should be used for the shortest duration possible. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically, especially when treatment continues for more than 4 weeks.
Patients should remain alert for the signs and symptoms of serious arteriothrombotic events (e.g. chest pain, shortness of breath, weakness, slurring of speech), which can occur without warnings. Patients should be instructed to see a physician immediately in case of such an event.

<u>Heart Failure</u>
Fluid retention and oedema have been observed in some patients taking NSAIDs, therefore caution is advised in patients with fluid retention or heart failure.

<u>Severe Skin Reactions</u>
Severe cutaneous reactions, including Stevens-Johnson Syndrome and toxic epidermal necrolysis (Lyell's Syndrome), have been reported with diclofenac sodium. Patients treated with diclofenac sodium should be closely monitored for signs of hypersensitivity reactions. Discontinue diclofenac sodium immediately if rash occurs. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

It should be given with care to patients with bleeding disorders, cardiovascular disease, peptic ulceration or a history of such ulceration. Patients who are sensitive to aspirin should generally not be given diclofenac sodium.
Patients with significantly impaired renal function should be closely monitored. A reduction in dosage should be anticipated to avoid drug accumulation. Diclofenac sodium inhibits platelets aggregation, thus prolonging bleeding time. It should therefore be used with caution in persons with intrinsic coagulation defects with those on anticoagulant therapy.

Risk of GI Ulceration, Bleeding and Perforation with NSAIDs
Serious GI Toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patient treated with NSAIDs therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.
Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Interactions with Other Medicaments:
Lithium/Digoxin: Diclofenac sodium may raise plasma concentrations of lithium and digoxin.
Diuretics: Like other NSAIDs, diclofenac sodium may decrease the activity of diuretics.
Concomitant treatment with potassium-sparing diuretics may be associated with increased serum potassium levels, which should therefore be monitored. NSAIDs: Concomitant administration of systemic NSAIDs may increase the frequency of adverse effects.
Anticoagulants: Although clinical studies do not appear to indicate that diclofenac sodium affects the action of anticoagulants, there are isolated reports of an increased risk of haemorrhage in patients receiving diclofenac sodium and anticoagulants concomitantly. Close monitoring of such patients is therefore recommended.
Antidiabetics: Clinical studies have shown that diclofenac sodium can be given concomitantly with oral antidiabetic agents without influencing their clinical effect. However, isolated cases have been reported of hypo and hyperglycaemic reactions necessitating changes in the dosage of hypoglycaemic agents during treatment with diclofenac sodium.
Methotrexate: Caution is called for if NSAIDs are administered < 24 hrs before or after treatment with methotrexate, since methotrexate levels in the blood may rise and the toxicity of methotrexate be increased.
Cyclosporin: The effect of NSAIDs on renal prostaglandins may increase the nephrotoxicity of cyclosporin.
Quinolone Antibacterial: There have been isolated reports of convulsions which may have been due to concomitant use of quinolones and NSAIDs.

Pregnancy and Lactation:
Because of the known effects of non-steroidal anti-inflammatory agents on foetal cardiovascular system (closure of ductus arteriosus), use in late pregnancy should be avoided.
Use of NSAIDs at about 20 weeks gestation or later in pregnancy may cause foetal renal dysfunction leading to oligohydramnios and in some cases, neonatal renal impairment. These adverse outcomes are seen, on average, after days to weeks of treatment, although oligohydramnios has been infrequently reported as soon as 48 hours after NSAID initiation. Oligohydramnios is often, but not always, reversible with treatment discontinuation.
Because of the possible side-effects of prostaglandin inhibiting drugs on neonates, diclofenac sodium is not recommended for use in nursing mothers.

Side Effects:
<u>Dermatological</u>
Occasional: Rashes or skin eruptions.
Cases of hair loss, bullous eruptions, erythema multiforme. Stevens-Johnson Syndrome, toxic epidermal necrolysis (Lyell's syndrome) and photosensitivity reactions have been reported. Others include gastrointestinal disturbances, pruritus, dizziness, malaise, headache, drowsiness, jaundice and bleeding tendency.

<u>Cardiac Disorders</u>
Kounis syndrome: Frequency "not known"
Uncommon*: Myocardial infarction, cardiac failure, palpitations, chest pain.
*The frequency reflects data from long-term treatment with a high dose (150 mg/day).

<u>Description of selected adverse drug reactions</u>
Arteriothrombotic Events
Meta-analysis and pharmacoepidemiological data point towards an increased risk of arteriothrombotic events (for example myocardial infarction) associated with the use of diclofenac, particularly at a high dose (150 mg daily) and during long-term treatment (see section WARNINGS AND PRECAUTIONS).

Symptoms and Treatment of Overdose:
Symptoms include apnea, gastro-intestinal disturbances, confusion, dizziness and headache.
Treatment: The stomach should be emptied by vomiting or lavage. Because the drug is acidic and is mainly excreted in the urine, it may be beneficial to administer alkali and induce diuresis.

Effects on Ability to Drive and Use Machine: None known.

Storage Conditions:
Fenadium Tablets 25mg : Store at or below 30ºC.
Fenadium F Tablets 50mg : Store at or below 30°C.

Pack Size:
Fenadium Tablets 25mg:
Blister pack: A box of 10 x 10, 50 x 10 and 100 x 10 tablets per strip.
Fenadium F Tablets 50mg:
Blister pack: A box of 10 x 10, 50 x 10 and 100 x 10 tablets per strip.

Pack Size (export only):
Fenadium Tablets 25mg : A bottle of 1000 tablets.
Fenadium F Tablets 50mg : A bottle of 90, 100 and 1000 tablets.

Shelf-life:
Fenadium Tablets 25mg: Blister pack: 3 years; Bottle pack: 4 years.
Fenadium F Tablets 50mg: 5 years.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:
Sunward Pharmaceutical Sdn. Bhd.
No. 9, 11 & 17, Jalan Kempas 4,
Taman Perindustrian Tampoi Indah,
81200 Johor Bahru, Johor, Malaysia.

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